



MINDFUL DIABETES FREE GUIDES

Simple, practical guidance for metabolic and brain health.

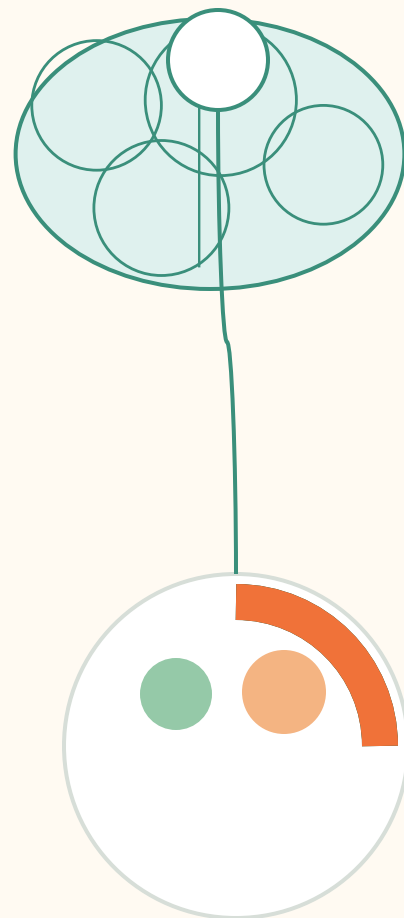
FREE GUIDE

Blood Sugar & Brain Health

Understanding the Everyday Connection

A calm guide to connected care

Glucose, blood vessels, daily routines, sensory health, and clinical care all fit into the same bigger conversation.



Mindful Diabetes Inc. | 501(c)(3) nonprofit

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MISSION

The everyday connection

This guide is for people with and without diabetes, families, caregivers, and educators.

Brain health is not controlled by one number or one habit. It is shaped by a wider pattern of metabolic health, vascular health, sensory access, sleep, mood, connection, and qualified care. [4,6-8]



Risk can sometimes be influenced, but it cannot be perfectly controlled.

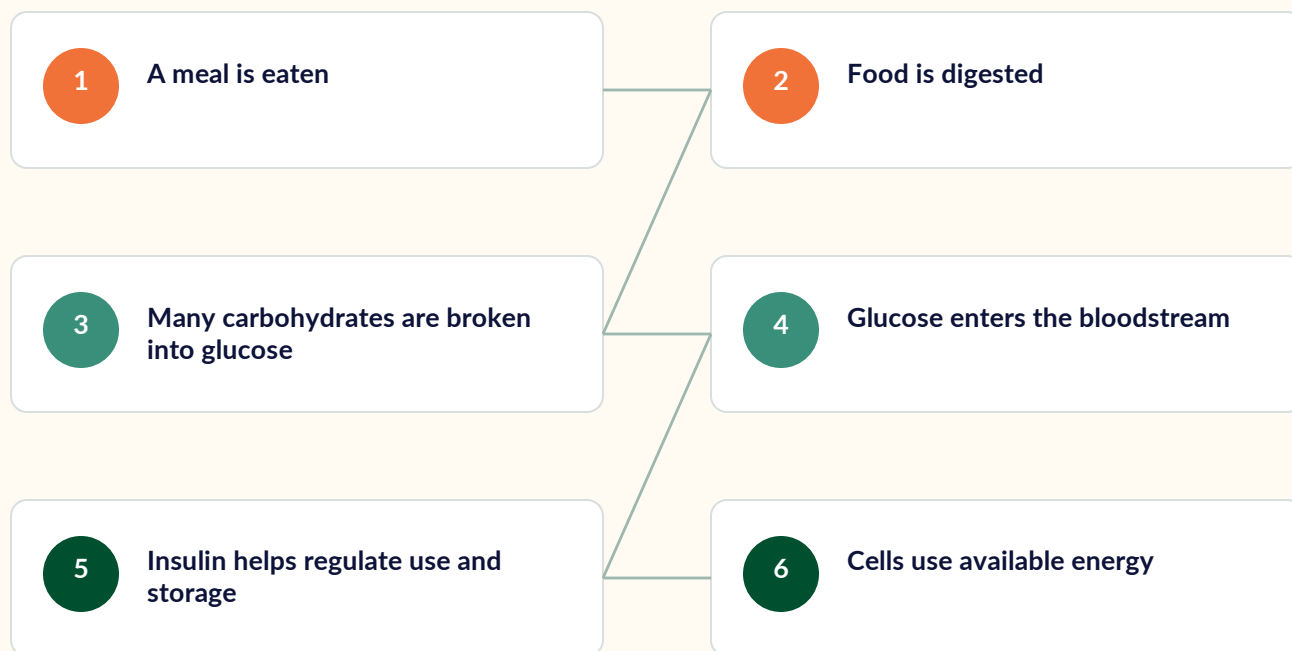
Use this guide to organize questions, not to predict the future. Memory concerns deserve qualified evaluation.

GLUCOSE BASICS

What glucose does

Glucose is one useful energy source. Diabetes care is not about eliminating glucose.

During digestion, many carbohydrate-containing foods are broken into glucose, which enters the bloodstream. Fiber is not fully digested into glucose, and different carbohydrate foods can affect people differently. [1,2,10]



The brain needs more than glucose.

Thinking and memory also depend on oxygen, blood flow, sleep, medications, sensory access, mood, and broader health. [4,6]

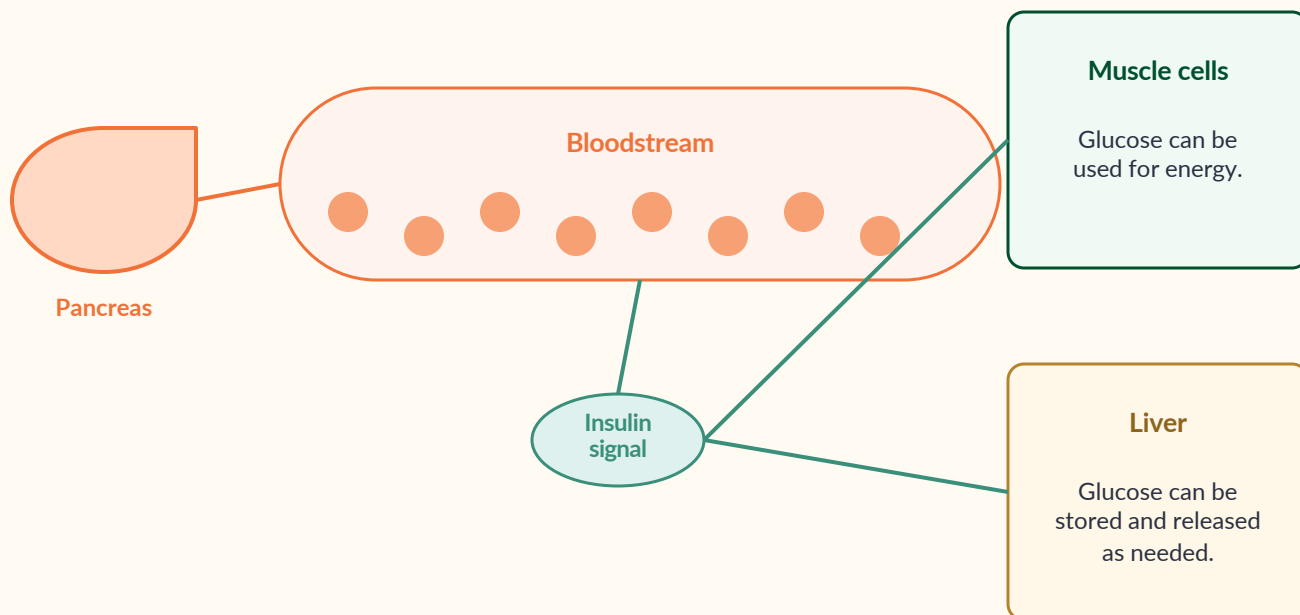
INSULIN

How insulin helps regulate glucose

Insulin is a hormone. For some people, it is also a prescribed medicine.

Insulin helps regulate how glucose moves from the bloodstream into cells and how the body stores energy in tissues such as muscle and liver. Personal glucose targets vary; medicine decisions belong with the care team.

[1,2]



What is personal

Glucose targets vary by age, diabetes type, medicines, pregnancy status, health history, and hypoglycemia risk.

A target range is a care-plan decision, not a moral score.

What insulin is not

Insulin is not a punishment or failure.

People with diabetes do not all have the same insulin needs or the same treatment plan.

INSULIN RESISTANCE

Insulin resistance is biology, not blame

Certain tissues may respond less effectively to insulin signals.

1

Insulin sends a signal.

2

Certain tissues respond less effectively.

3

The body may produce more insulin for a time.

4

Glucose may remain elevated when compensation is not enough.

5

Clinical support and sustainable changes may help manage risk.

Not a character judgment

Insulin resistance is influenced by biology, health conditions, medicines, sleep, movement, genetics, aging, and other factors. One food or one habit does not explain the whole picture. [1,17]

Helpful questions are practical: What patterns matter for me? What care-plan changes are appropriate? What support is realistic in my life?



Avoid overpromising

No routine, food, supplement, or single number guarantees reversal of insulin resistance or prevention of dementia.



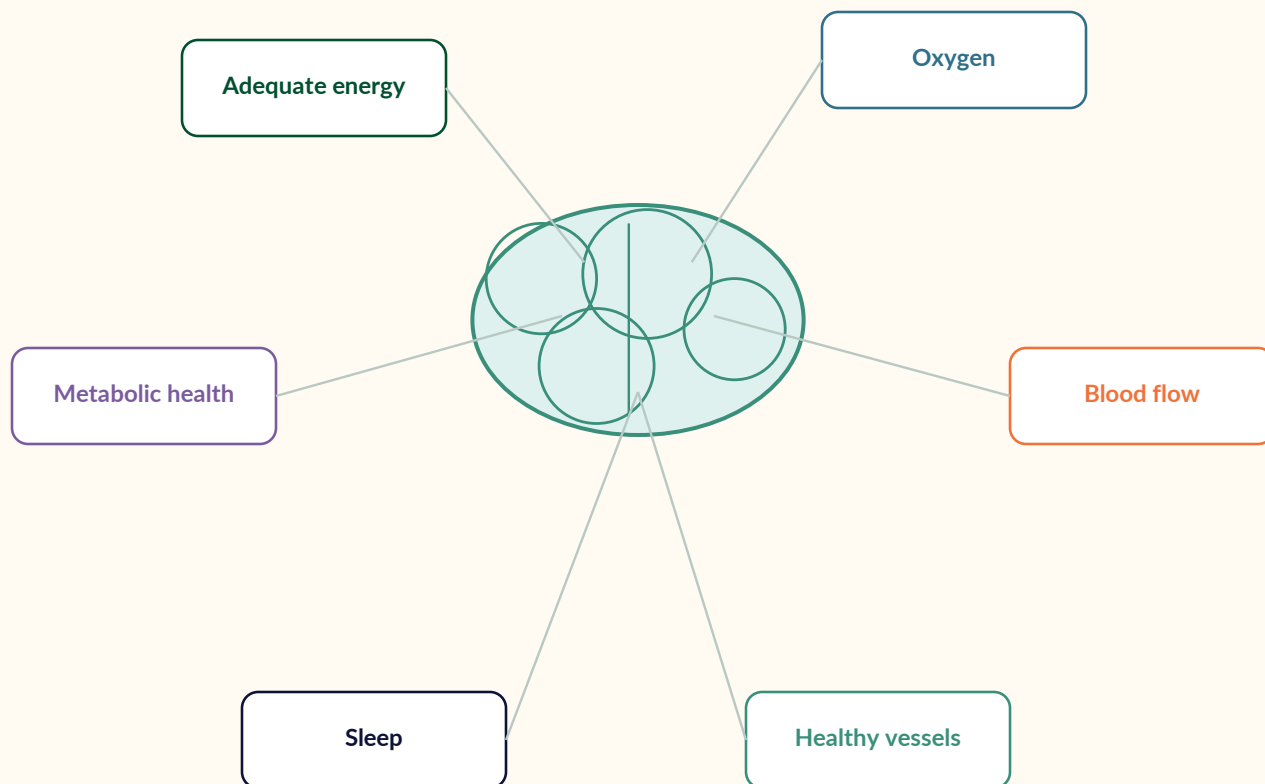
Use care support

Care teams can interpret labs, medicines, sleep, activity, symptoms, and goals together.

BRAIN SUPPORT

The brain needs reliable support

Brain function depends on energy delivery, oxygen, blood flow, sleep, and broader health.



● Sudden low glucose

Can affect attention, speech, judgment, coordination, or consciousness quickly.
Can become urgent; follow the personal low-glucose plan.

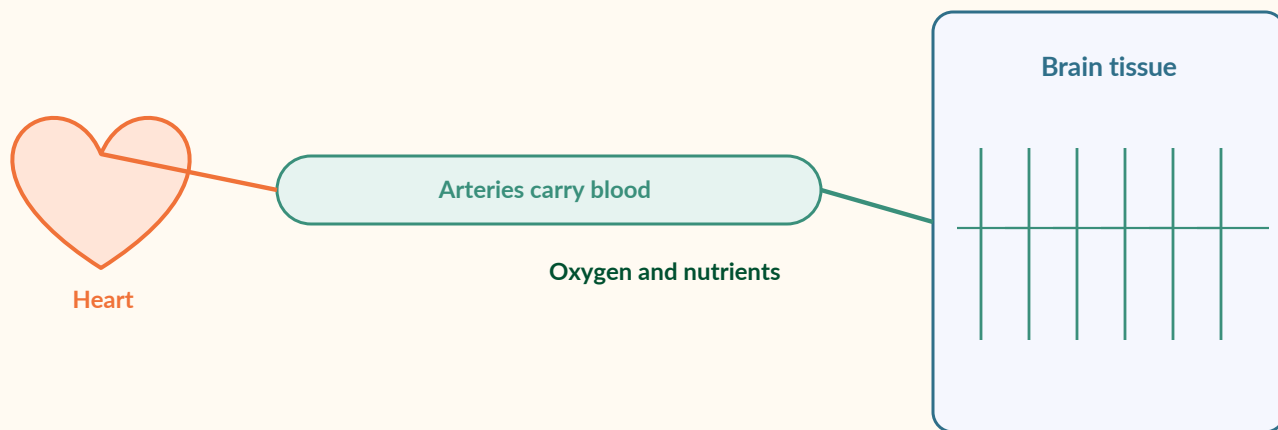
● High glucose patterns

May contribute to complications over time depending on severity, duration, and individual context.
Patterns deserve support, not shame.

BLOOD VESSELS

Blood vessels deliver what brain tissue needs

Vascular health is one part of cognitive-health care; it is not a diagnosis.



Blood pressure, diabetes, cholesterol, smoking, physical activity, and other clinical or environmental factors can affect vascular health. [4,8,9]

Vascular cognitive impairment is not the same thing as ordinary forgetfulness.

Potentially urgent

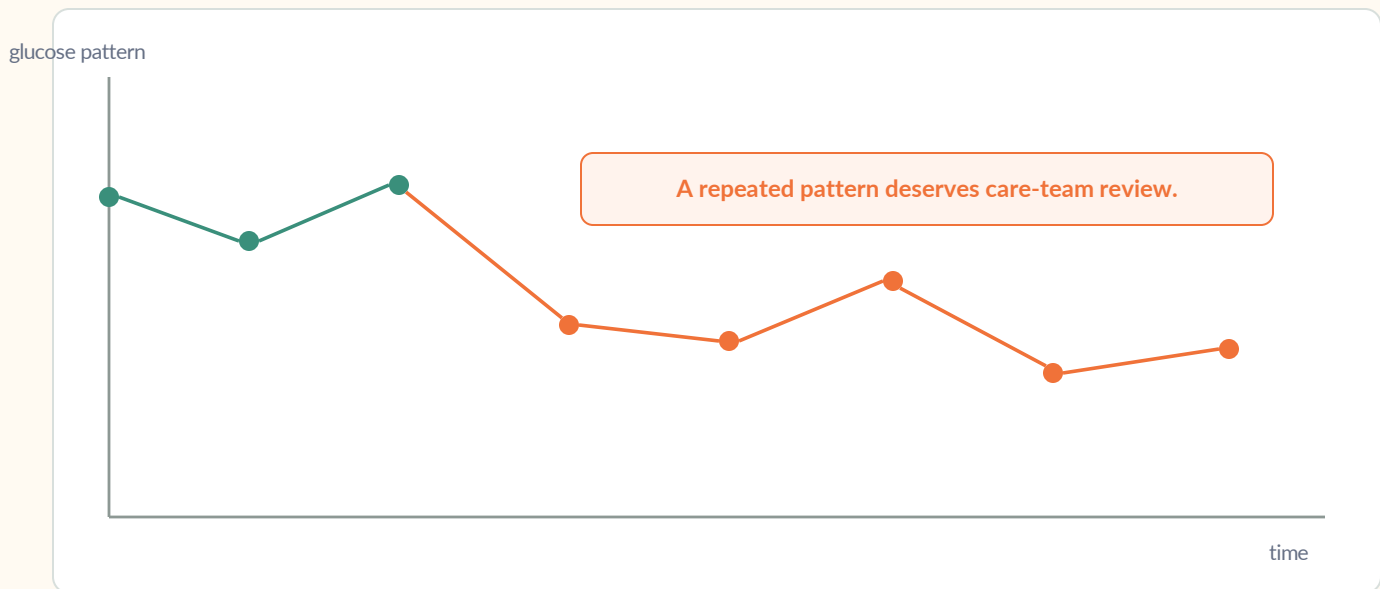
Sudden confusion, new weakness, facial drooping, trouble speaking, severe headache, chest pain, or stroke-like symptoms should be treated as urgent.

HYPERGLYCEMIA

High blood sugar over time

Patterns are information, not grades.

One isolated reading does not define a person. Repeated or prolonged high glucose may contribute to vascular and nerve complications over time, depending on severity, duration, and individual circumstances. [2,4]



Bring the pattern

What do these readings mean in my care plan?
Could medicines, illness, sleep, stress, food timing, or activity be involved?

Keep it nonjudgmental

Do not change medicines from this guide.
Ask what symptoms or patterns should prompt a call.

HYPOGLYCEMIA SAFETY

Low blood sugar can affect the brain quickly

Symptoms vary. People using insulin or certain medicines may be at greater risk. [2,3]

Early or common warning symptoms

- shaking
- sweating
- hunger
- dizziness

Thinking and coordination symptoms

- confusion
- behavior change
- difficulty speaking
- weakness

Severe symptoms

- seizure
- fainting
- inability to safely self-treat
- loss of consciousness

Safety actions

Follow your individual low-glucose plan. Do not drive during significant symptoms. Severe confusion, seizure, fainting, or inability to safely self-treat requires appropriate emergency assistance.

COGNITIVE RISK

Diabetes is associated with increased risk; it is not destiny

Dementia has multiple causes and contributors. Risk factors can interact across a life course. [5,7,8]



Potentially modifiable or treatable

- Blood pressure
- Cholesterol and vascular risk
- Smoking and excessive alcohol use
- Physical inactivity
- Hearing or vision concerns
- Depression and social isolation

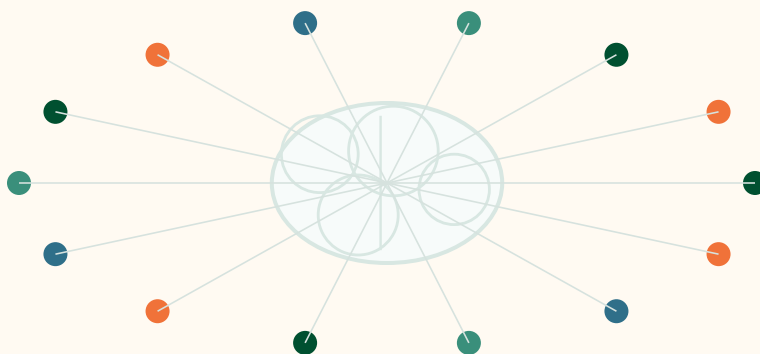


Not fully controllable

- Age
- Genetics and family history
- Early-life conditions
- Environmental exposures
- Access to care
- Existing medical history

Important distinction

Population-level association does not mean every person with diabetes will develop dementia. Memory or thinking changes deserve professional evaluation.



RISK MEANING

Risk information should guide questions, not panic

A risk estimate describes patterns in groups; it does not predict one person's future with certainty. [6-8]

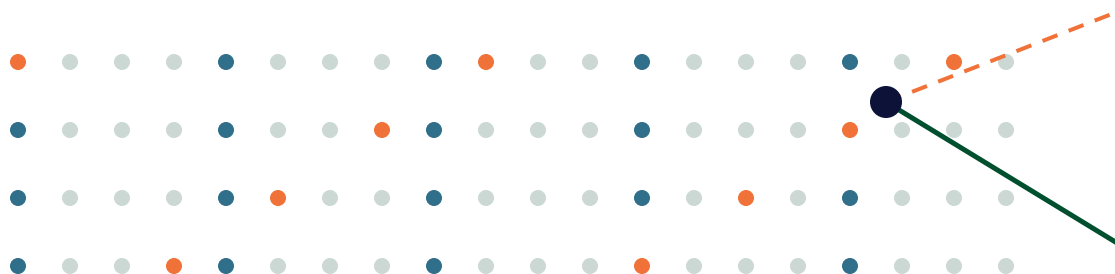
● Group-level risk

Describes patterns across many people.
Supports public-health planning.
Helps identify areas for clinical attention.

● Individual future

Cannot be predicted from one factor.
Depends on many interacting circumstances.
Requires personal medical context.

Hypothetical: many paths, no single forecast



Group patterns

One person still needs personal context.

A healthier use of risk

Ask what applies to you, what can be evaluated, and what support is realistic. Avoid using group statistics as a personal forecast.

VASCULAR CARE

Blood pressure, cholesterol, and glucose are interpreted together

Targets vary by age, medicines, health history, and overall cardiovascular risk. [2,9,17]

Blood pressure

Ask what target applies.

Lipids

Ask which results matter most.

Glucose or A1C

Use clinician-selected measures.

Medicines

Discuss benefits and side effects.

Smoking status

Ask about support without shame.

Family history

Use it to guide questions.

The care question is not "Which number is good or bad?"

It is "What do my numbers mean together, and what is the next appropriate step for me?"

Do not change treatment from this guide.

Medication decisions belong with qualified care. Bring questions about targets, side effects, affordability, and follow-up.

MOVEMENT

Physical activity can be flexible and mobility-inclusive

Activity may support cardiovascular health, metabolic health, mood, sleep, daily function, and quality of life. [11,17]

 Walking

Short, safe bouts can count.

 Seated movement

Chair routines can support circulation.

 Standing breaks

Small interruptions to sitting may help.

 Strength activity

Use bands, weights, or body weight if appropriate.

 Stretching

Can support comfort and function.

 Physical therapy

Clinical guidance can adapt movement safely.

 Household movement

Daily tasks can be part of an activity pattern.

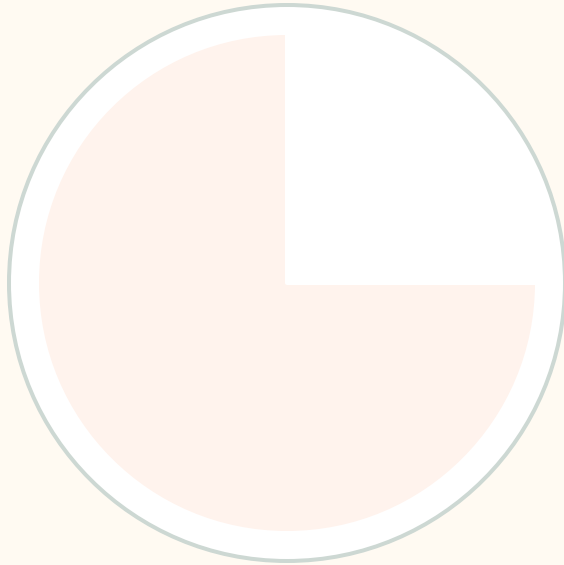
My safest realistic movement option: _____

Walking is one option, not a requirement. Ask about safe options if pain, balance, fatigue, disability, or recovery makes movement complicated.

FOOD PATTERNS

Food patterns matter more than brain-superfood claims

mixed dishes, culture, foods, access, budget, and medical nutrition needs all matter. [10,17]



Flexible plate visual

**Vegetables and fruit**

fresh, frozen, canned, cooked into dishes

**Beans, lentils, whole grains**

carbohydrate foods can fit a care plan

**Plant or lean proteins**

fish where appropriate, tofu, eggs, poultry, legumes

**Nuts, seeds, unsaturated fats**

flavor and satiety without cure claims

**Lower-sugar drink options**

water, unsweetened drinks, or personal alternatives

**Culture and access**

a useful pattern should fit real life

No single food is a brain-health cure.

The plate method is a flexible visual tool, not a prescription. Medical nutrition needs vary; carbohydrate removal is not the goal of brain health.

SLEEP

Sleep problems deserve care, not blame

Sleep can interact with mood, attention, glucose patterns, blood pressure, decision-making, and quality of life. [12,17]



Life can complicate sleep

- Shift work
- Caregiving
- Pain
- Medicines
- Insomnia
- Restless legs

Ask about evaluation

- Loud snoring
- Breathing pauses
- Gasping during sleep
- Daytime sleepiness
- Morning headaches
- Sleep that stays unrefreshing

Sleep hygiene is not a full treatment plan.

Breathing pauses, severe insomnia, restless legs, medication effects, pain, and caregiving stress may need clinical support.

TOBACCO AND ALCOHOL

Support works better than shame

Tobacco avoidance and alcohol decisions fit within overall cardiovascular, metabolic, and cognitive-health care. [7,8,17,18]



Tobacco

Avoiding tobacco supports overall and cardiovascular health.
Quitting can require repeated attempts.
Counseling, medicines, and practical support may help.
Current tobacco use is not a character flaw.



Alcohol

Excessive use can create health risks.
Alcohol may interact with medicines and glucose risk.
People who do not drink should not start for health.
Individual guidance may be appropriate.

A calmer question

What kind of support would make the next step more realistic: counseling, medication options, a safety plan around alcohol and glucose, or a referral?

No universal safe amount is set here.

This guide does not define a personal alcohol limit or recommend medication changes.

COMMUNICATION AND CONNECTION

Hearing, vision, mood, and connection affect daily function

These factors may affect communication, cognitive load, safety, quality of life, and access to care. [13-16]

Hearing

Ask about testing when communication changes. Improve access with quieter rooms, captions, hearing devices, or face-to-face conversation.

Vision

Address correctable or treatable problems. Diabetes eye care can support safe daily function.

Mood

Depression deserves assessment and care. Mood changes should not be dismissed as just aging.

Social connection

Connection can be family, friends, neighbors, faith, support groups, community programs, or routine contact.

Nuance matters

Treating hearing, vision, mood, or isolation may reduce strain and improve function. No single intervention guarantees dementia prevention.

WORKSHEET

Questions for your clinician

Bring the guide or a completed worksheet if it helps organize the visit. Leave blanks if something does not apply.

- | | |
|---|---|
| ☐ My A1C or glucose targets | ☐ Low-glucose risk |
| ☐ Blood pressure | ☐ Cholesterol |
| ☐ Medicines and side effects | ☐ Memory or thinking changes |
| ☐ Sleep | ☐ Hearing |
| ☐ Vision | ☐ Mood |
| ☐ Family history | ☐ Referrals or testing |

My most important question**What I want to understand before leaving****Next step or referral discussed****Privacy reminder**

This page may include personal health information. Store completed worksheets in a place you choose.

WORKSHEET

Family-history worksheet

Family history can guide questions. It does not determine your future.

Write what you know. Leave blanks where you are unsure. Approximate ages, relationships, and uncertainty are acceptable.

Condition or concern	Relative	Side if known	Approx. age	What I know / what I am unsure about	Question for my clinician
Diabetes					
Stroke					
High blood pressure					
High cholesterol					
Heart disease					
Dementia or significant memory problems					

Privacy reminder

Completed family-history pages may contain private information. Share only with people you choose.

ORGANIZER

Health Numbers and Care-Plan Organizer

Use this page to keep clinician-selected targets and next steps in one place.

Do not change treatment based on this organizer. Ask your care team which numbers matter for you and how often to review them.

Appointment 1

Date

A1C or glucose measure selected by clinician

Blood pressure

Lipid result or clinician

Medicine changes made by care team

Sleep, activity, or symptom note

Target or next step

Appointment 2

Date

A1C or glucose measure selected by clinician

Blood pressure

Lipid result or clinician

Medicine changes made by care team

Sleep, activity, or symptom note

Target or next step

Appointment 3

Date

A1C or glucose measure selected by clinician

Blood pressure

Lipid result or clinician

Medicine changes made by care team

Sleep, activity, or symptom note

Target or next step

Privacy reminder

Store completed health information securely. It is okay to leave fields blank.

CAREFUL HOPE

What risk reduction can and cannot promise

Healthy routines and medical care may support health, but they do not guarantee a specific cognitive outcome. [6-8,17]

● May support

- Vascular health
- Daily function
- Better clinical questions
- Identification of treatable problems
- Risk reduction in a broader care plan
- Quality of life

● Cannot guarantee

- Prevention of Alzheimer's disease
- Prevention of every dementia
- Reversal of every diabetes complication
- A specific cognitive outcome
- Replacement for screening or care
- Perfect control over risk

Use careful hope.

Care can be meaningful even when it cannot promise certainty.

The goal is not panic.

Use the guide to plan questions, notice concerning changes, and build support that fits your life.

RESOURCES

Next steps

Use this guide to prepare questions rather than predict the future.

Share memory or thinking concerns with qualified care. [6]

Follow your personal glucose plan and low-glucose instructions. [2,3]

Ask what blood-pressure and cholesterol targets apply to you. [9]

Address hearing, vision, sleep, and mood concerns when they affect daily life. [12,14-16]

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Medical disclaimer: This guide is general education. It does not diagnose, treat, or replace qualified medical care.

Urgent concerns

Sudden confusion, stroke-like symptoms, severe low glucose symptoms, fainting, seizure, chest pain, or major changes require appropriate urgent assistance.

REFERENCES AND SAFETY

Safety notes and references

Medical review status: pending | Next scheduled review: July 2027 | Accessibility status: checked, not PDF/UA certified.

This guide is education, not diagnosis.

It does not provide personal glucose targets, medicine changes, dementia diagnosis, or emergency instructions beyond encouraging appropriate urgent help. Discuss individual questions with qualified care.

Potentially urgent

Sudden confusion, stroke-like symptoms, severe low glucose symptoms, fainting, seizure, loss of consciousness, chest pain, or trouble breathing should receive appropriate urgent assistance.

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- [2] American Diabetes Association Professional Practice Committee for Diabetes. 6. Glycemic Goals, Hypoglycemia, and Hyperglycemic Crises: Standards of Care in Diabetes - 2026. Diabetes Care. 2026;49(Suppl. 1):S132-S149. doi:10.2337/dc26-S006. https://diabetesjournals.org/care/article/49/Supplement_1/S132/163927/6-Glycemic-Goals-Hypoglycemia-and-Hyperglycemic
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- [5] American Diabetes Association Professional Practice Committee for Diabetes. 13. Older Adults: Standards of Care in Diabetes - 2026. Diabetes Care. 2026;49(Suppl. 1):S277-S296. doi:10.2337/dc26-S013. https://diabetesjournals.org/care/article/49/Supplement_1/S277/163921/13-Older-Adults-Standards-of-Care-in-Diabetes-2026
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All in-text citations use sequential numbers that appear in this list.

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[13] Centers for Disease Control and Prevention. Health Effects of Social Isolation and Loneliness. Updated May 15, 2024. <https://www.cdc.gov/social-connectedness/risk-factors/index.html>

[14] National Eye Institute. Diabetic Retinopathy. Updated September 2025. <https://www.nei.nih.gov/eye-health-information/eye-conditions-and-diseases/diabetic-retinopathy>

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[17] American Diabetes Association Professional Practice Committee for Diabetes. 5. Facilitating Positive Health Behaviors and Well-being to Improve Health Outcomes: Standards of Care in Diabetes - 2026. Diabetes Care. 2026;49(Suppl. 1):S89-S131. doi:10.2337/dc26-S005. https://diabetesjournals.org/care/article/49/Supplement_1/S89/163932/5-Facilitating-Positive-Health-Behaviors-and-Well

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Citation check

No [12]-[15] orphan citation pattern remains from the draft. Each body citation points to a numbered source in the final list; no references are unused.